

GRAIN DIRECTION



SAME SIZE ARTWORK
LEAFLET: 225 mm x 270 mm
FOLDING SIZE: 38 mm x 135 mm

38 mm

135 mm

SPACE FOR PHARMACODE

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For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

Combiwave[®]

SF 125

Salmeterol 25mcg / actuation and Fluticasone Propionate 125mcg / actuation Pressurised Inhalation

Combiwave[®]

SF 250

Salmeterol 25mcg / actuation and Fluticasone Propionate 250mcg / actuation Pressurised Inhalation

COMPOSITION

Each actuation delivers:

Salmeterol Xinafoate BP equivalent to Salmeterol 25 mcg

Fluticasone Propionate BP 125 mcg

Excipients & Propellant 1,1,1,2 Tetrafluoro Ethane (HFA 134a) q.s

Combiwave SF 250

Each actuation delivers:

Salmeterol Xinafoate BP equivalent to Salmeterol 25 mcg

Fluticasone Propionate BP 250mcg

Excipients & Propellant 1,1,1,2 Tetrafluoro Ethane (HFA 134a) q.s.

DOSAGE FORM

Inhalation aerosol

DESCRIPTION

White to off white suspension filled in aluminium aerosol can with silver colour ferrule valve provided with actuator suitable for oral inhalation when actuated a metered spray is produced.

PHARMACOLOGY

Pharmacodynamics

Since Salmeterol and Fluticasone Propionate Pressurized Inhalation contains both fluticasone propionate and salmeterol, the mechanism of action described below for the individual components apply to Salmeterol and Fluticasone Propionate Pressurized Inhalation. These drugs represent two classes of medications (a synthetic corticosteroid and a selective, long-acting beta 2-adrenergic receptor agonist) that have different effects on the clinical, physiologic, and inflammatory indices of asthma.

Fluticasone Propionate:

Fluticasone propionate is a synthetic, trifluorinated corticosteroid with potent anti-inflammatory activity. The precise mechanisms of fluticasone propionate action in asthma are unknown. Inflammation is an important component in the pathogenesis of asthma. Corticosteroids have been shown to inhibit multiple cell types (e.g., mast cells, eosinophils, basophils, lymphocytes, macrophages, and neutrophils) and mediator production or secretion (e.g., histamine, eicosanoids, leukotrienes, and cytokines) involved in the asthmatic response. These anti-inflammatory actions of corticosteroids contribute to their efficacy in asthma.

Salmeterol:

Salmeterol is a long-acting beta2-adrenergic agonist. Although beta2-adrenoceptors are the predominant adrenergic receptors in bronchial smooth muscle and beta1-adrenoceptors are the predominant receptors in the heart, there are also beta2-adrenoceptors in the human heart, comprising 10-50% of the total beta-adrenoceptors. The precise function of these receptors has not been established, but their presence raises the possibility that even selective beta2-agonists may have cardiac effects. The pharmacologic effects of beta2-adrenoceptor agonist drugs, including salmeterol, are at least in part attributable to stimulation of intracellular adenylyl cyclase, the enzyme that catalyses the conversion of adenosine triphosphate (ATP) to cyclic-3', 5'-adenosine monophosphate (cyclic AMP). Increased cyclic AMP levels cause relaxation of bronchial smooth muscle and inhibition of release of mediators of immediate hypersensitivity from cells, especially from mast cells. Salmeterol contains a long side chain, which binds to the exo-site of the receptor.

Pharmacokinetics

Salmeterol, a long acting β_2 -agonist which acts locally in the lung to mediate bronchodilation.

Fluticasone, a corticosteroid with mainly glucocorticoid activity, reduce symptoms and exacerbations of asthma.

Onset: Bronchodilation: 10-20 minutes.

Duration: Bronchodilation: 12 hr

Absorption: Salmeterol: Negligible absorption after inhalation. Fluticasone: Poorly absorbed from the GI tract; oral bioavailability <1%; absolute bioavailability of inhaled fluticasone: 5-11% (depending on device used).

Distribution: Salmeterol: Protein binding: 96%. Fluticasone: Protein binding: 91%.

Metabolism: Salmeterol: Extensive hepatic metabolism by hydroxylation; terminal elimination half-lives: 5.5 hr. Fluticasone: Extensive first-pass metabolism by cytochrome CYP3A4.

Excretion: Salmeterol: Eliminated mainly in faeces; negligible amounts of unchanged salmeterol are detectable in urine or faeces. Fluticasone: Mainly excreted in faeces as metabolites and unchanged drug; <5% excreted in urine.

INDICATIONS

Reversible Obstructive Airways Disease (ROAD): Salmeterol and Fluticasone Propionate Pressurized Inhalation is indicated in the regular treatment of reversible obstructive airways disease (ROAD), including asthma in children and adults, where use of a combination (bronchodilator and inhaled corticosteroid) is appropriate.

This may include:

- Patients on effective maintenance doses of long-acting beta agonists and inhaled corticosteroids.
- Patients who are symptomatic on current inhaled corticosteroid therapy.
- Patients on regular bronchodilator therapy who require inhaled corticosteroids.

Chronic Obstructive Pulmonary Disease (COPD): Salmeterol and Fluticasone Propionate Pressurized Inhalation is indicated for the regular treatment of chronic obstructive pulmonary disease (COPD) including chronic bronchitis and emphysema.

DOSAGE AND ADMINISTRATION

Combiwave SF 125 is for inhalation only. Patients should be made aware that Combiwave SF 125 must be used regularly for optimum benefit, even when asymptomatic. Patients should be regularly reassessed by a doctor, so that the strength of Combiwave SF 125 they are receiving remains optimal and is only changed on medical advice.

Reversible Obstructive Airways Disease (ROAD)

The dose should be titrated to the lowest dose at which effective control of symptoms is maintained. Where the control of symptoms is maintained with twice daily Combiwave SF 125, titration to the lowest effective dose could include Combiwave SF 125 given once daily. If a patient is inadequately controlled on inhaled corticosteroid therapy alone, substitution with Combiwave SF 125 at a therapeutically equivalent corticosteroid dose may result in an improvement in asthma control. For patients whose asthma control is acceptable on inhaled corticosteroid therapy alone, substitution with Combiwave SF 125 may permit a reduction in corticosteroid dose while maintaining asthma control. For further information, please refer to the 'Pharmacodynamics' section.

Recommended Doses: Adults and adolescents 12 years and older: - Two inhalations of 25 micrograms salmeterol and 125 micrograms fluticasone propionate twice daily.

Chronic Obstructive Pulmonary Disease (COPD)

For adult patients the recommended dose is two inhalations 25/125 micrograms salmeterol/fluticasone propionate twice daily.

Special patient groups:- There is no need to adjust the dose in elderly patients or in those with renal or hepatic impairment.

CONTRAINDICATIONS

Salmeterol and Fluticasone Propionate Pressurized Inhalation is contraindicated in patients with a history of hypersensitivity to any of the component of the product.

WARNINGS AND PRECAUTIONS

The management of ROAD should normally follow a stepwise programme and patient response should be monitored clinically and by lung function tests.

Combiwave SF 125 should not be used to treat acute asthma symptoms for which a fast and short acting bronchodilator is required. Patients should be advised to have their medicinal product to be used for relief in an acute asthma attack available at all times.

Increasing use of short-acting bronchodilators to relieve symptoms indicates deterioration of control and patients should be reviewed by a physician.

Sudden and progressive deterioration in control of asthma is potentially life-threatening and the patient should be reviewed by a physician. Consideration should be given to increasing corticosteroid therapy. Also, where the current dosage of Combiwave SF 125 has failed to give adequate control of ROAD, the patient should be reviewed by a physician.

Combiwave SF 125 should not be initiated in patients with unstable and acutely deteriorating asthma, which may be a life-threatening condition. Serious acute respiratory events, including fatalities, have been reported when salmeterol has been initiated in this situation. Although it is not possible from these reports to determine whether salmeterol contributed to these adverse events or failed to relieve the deteriorating asthma, the use of salmeterol in this setting is inappropriate.

Combiwave SF 125 should not be used to transfer patients from oral to inhaled steroids. For patients with asthma or COPD, consideration should be given to additional corticosteroid therapies and administration of antibiotics if an exacerbation is associated with infection.

Treatment with Combiwave SF 125 should not be stopped abruptly in patients with asthma due to risk of exacerbation; therapy should be titrated-down under physician supervision. For patients with COPD cessation of therapy may be associated with symptomatic decompensation and should be supervised by a physician.

As with all inhaled medication containing corticosteroids, Combiwave SF 125 should be administered with caution in patients with active or quiescent pulmonary tuberculosis.

Combiwave SF 125 should be administered with caution in patients with thyrotoxicosis.

Cardiovascular effects, such as increases in systolic blood pressure and heart rate, may occasionally be seen with all sympathomimetic drugs, especially at higher than therapeutic doses. For this reason, Combiwave SF 125 should be used with caution in patients with pre-existing cardiovascular disease.

A transient decrease in serum potassium may occur with all sympathomimetic drugs at higher therapeutic doses. Therefore, Combiwave SF 125 should be used with caution in patients predisposed to low levels of serum potassium.

Systemic effects may occur with any inhaled corticosteroid, particularly at high doses prescribed for long periods; these effects are much less likely to occur than with oral corticosteroids. Possible systemic effects include Cushing's syndrome, Cushingoid features, adrenal suppression, growth retardation in children and adolescents decrease in bone mineral density, cataract and glaucoma.

It is important, therefore for ROAD patients, that the dose of inhaled corticosteroid is titrated to the lowest dose at which effective control is maintained.

The possibility of impaired adrenal response should always be borne in mind in emergency and elective situations likely to produce stress and appropriate corticosteroid treatment considered.

It is recommended that the height of children receiving prolonged treatment with inhaled corticosteroid is regularly monitored.

Certain individuals can show greater susceptibility to the effects of inhaled corticosteroids than do most patients.

Because of the possibility of impaired adrenal response, patients transferring from oral steroid therapy be treated with special care, and adrenocortical function regularly monitored.

Following introduction of inhaled fluticasone propionate, withdrawal of systemic therapy should be gradual and patients encouraged to carry a steroid warning card indicating the possible need for additional therapy in times of stress.

There have been very rare reports of increases in blood glucose levels and this should be considered when prescribing to patients with a history of diabetes mellitus.

Ritonavir (a highly potent cytochrome P450 3A4 inhibitor) can greatly increase fluticasone propionate plasma concentrations, resulting in markedly reduced serum cortisol concentrations. There have been reports of clinically significant drug interactions in patients receiving fluticasone propionate and ritonavir, resulting in systemic corticosteroid effects including cushing's syndrome and adrenal suppression. Therefore, concomitant use of fluticasone propionate and ritonavir should be avoided, unless the potential benefit to the patient outweighs the risk of systemic corticosteroid side-effects.

Pneumonia in patients with COPD

An increase in the incidence of pneumonia, including pneumonia requiring hospitalisation, has been observed in patients with COPD receiving inhaled corticosteroids. There is some evidence of an increased risk of pneumonia with increasing steroid dose but this has not been demonstrated conclusively across all studies.

There is no conclusive clinical evidence for intra-class differences in the magnitude of the pneumonia risk among inhaled corticosteroid products. Physicians should remain vigilant for the possible development of pneumonia in patient with COPD as the clinical features of such infections overlap with the symptoms of COPD exacerbations.

Risk factors for pneumonia in patients with COPD include current smoking status, older age, low body mass index (BMI) and severe COPD.

Drug Interactions

Co-treatment with CYP3A4 inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side-effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects, in which case patients should be monitored for systemic corticosteroid side-effects.

Both non-selective and selective beta-blockers should be avoided in patients with asthma, unless there are compelling reasons for their use.

Under normal circumstances, low plasma concentrations of fluticasone propionate are achieved after inhaled dosing, due to extensive first pass metabolism and high systemic clearance mediated by cytochrome P450 3A4 in the gut and liver. Hence, clinically significant drug interactions mediated by fluticasone propionate are unlikely.

In an interaction study in healthy subjects with intranasal fluticasone propionate, ritonavir (a highly potent cytochrome P450 3A4 inhibitor) can greatly increase fluticasone propionate plasma concentrations, resulting in markedly reduced serum cortisol concentrations. There have been reports of clinically significant drug interactions in patients receiving fluticasone propionate and ritonavir, resulting in systemic corticosteroid effects including Cushing's syndrome and adrenal suppression. Therefore the combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects.

Other inhibitors of cytochrome P4503A4 produce negligible (erythromycin) and minor (ketoconazole) increases in systemic exposure to fluticasone propionate without notable reductions in serum cortisol concentrations. Nevertheless, care is advised when co-administering potent cytochrome P4503A4 inhibitors (e.g. ketoconazole) as there is potential for increased systemic exposure to fluticasone propionate.

Pregnancy

Pregnancy Category C: There are insufficient data on the use of salmeterol and fluticasone propionate during pregnancy and lactation in man to assess the possible harmful effects.

Administration of Salmeterol and Fluticasone Propionate Pressurized Inhalation to pregnant women should only be considered if the expected benefit to the mother is greater than any possible risk to the foetus.

The lowest effective dose of fluticasone propionate needed to maintain adequate asthma control should be used in the treatment of pregnant women.

Lactation

There are no data available for human breast milk. Both salmeterol and fluticasone propionate are excreted into breast milk in rats. Administration of Salmeterol and Fluticasone Propionate Pressurized Inhalation to women who are breastfeeding should only be considered if the expected benefit to the mother is greater than any possible risk to the child.

UNDESIRABLE EFFECTS

As Combiwave SF 125 contains salmeterol and fluticasone propionate, the type and severity of adverse reactions associated with each of the compounds may be expected. There is no incidence of additional adverse events following concurrent administration of the two compounds.

Adverse events which have been associated with salmeterol/fluticasone propionate are given below, listed by system organ class and frequency. Frequencies are defined as: very common ($\geq 1/10$), common ($\geq 1/100$ to $<1/10$), uncommon ($\geq 1/1000$ to $<1/100$), rare ($\geq 1/10,000$ to $<1/1000$) and not known (cannot be estimated from the available data).

System Organ Class	Adverse Event	Frequency
Infections & Infestations	Candidiasis of the mouth and throat Pneumonia Bronchitis Oesophageal candidiasis	Common Common Common Rare
Immune System Disorders	Hypersensitivity reactions with the following manifestations: Cutaneous hypersensitivity reactions Angioedema (mainly facial and oropharyngeal oedema) Respiratory symptoms (dyspnoea) Respiratory symptoms (bronchospasm) Anaphylactic reactions including anaphylactic shock	Uncommon Rare Uncommon Rare Rare

PE00000 MY

ICONGRAPHICS CODE:

PANTONE SHADE

PANTONE
BLACK
PROCESS C

Supersedes
Artwork Code:

PHARMACODE:

GRAIN DIRECTION

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135 mm

25 mm

Endocrine Disorders	Cushing's syndrome, Cushingoid features, Adrenal suppression, Growth retardation in children and adolescents, Decreased bone mineral density	Rare
Metabolism & Nutrition Disorders	Hypokalaemia Hyperglycaemia	Common Uncommon
Psychiatric Disorders	Anxiety Sleep disorders Behavioural changes, including psychomotor hyperactivity and irritability (predominantly in children) Depression, aggression (predominantly in children)	Uncommon Uncommon Rare Not Known
Nervous System Disorders	Headache Tremor	Very Common Uncommon
Eye disorder	Cataract Glaucoma Vision, blurred	Uncommon Rare Not known
Cardiac Disorders	Palpitations Tachycardia Cardiac arrhythmias (including supraventricular tachycardia and extrasystoles). Atrial fibrillation Angina pectoris	Uncommon Uncommon Rare Uncommon Uncommon
Respiratory, Thoracic & Mediastinal Disorders	Nasopharyngitis Throat irritation Hoarseness/dysphonia Sinusitis Paradoxical bronchospasm	Very Common Common Common Common Rare
Skin and subcutaneous tissue disorders	Contusions	Common
Musculoskeletal & Connective Tissue Disorders	Muscle cramps Traumatic fractures Arthralgia Myalgia	Common Common Common Common

Description of selected adverse reactions
The pharmacological side effects of β_2 agonist treatment, such as tremor, palpitations and headache, have been reported, but tend to be transient and reduce with regular therapy.

As with other inhalation therapy paradoxical bronchospasm may occur with an immediate increase in wheezing and shortness of breath after dosing. Paradoxical bronchospasm responds to a rapid-acting bronchodilator and should be treated straightaway. This medicine should be discontinued immediately, the patient assessed and alternative therapy instituted if necessary.

Due to the fluticasone propionate component, hoarseness and candidiasis (thrush) of the mouth and throat and, rarely, of the oesophagus can occur in some patients. Both hoarseness and incidence of mouth and throat candidiasis may be relieved by rinsing the mouth with water and/or brushing the teeth after using the product. Symptomatic mouth and throat candidiasis can be treated with topical anti-fungal therapy whilst still continuing with this medicine.

Paediatric population
Possible systemic effects include Cushing's syndrome, Cushingoid features, adrenal suppression and growth retardation in children and adolescents. Children may also experience anxiety, sleep disorders and behavioural changes, including hyperactivity and irritability.

Reporting of suspected adverse reactions
Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions via the national reporting system.

OVERDOSAGE
The data on overdose with both drugs is given below:
The signs and symptoms of salmeterol overdose are tremor, headache and tachycardia. The preferred antidotes are cardioselective beta-blocking agents, which should be used with caution in patients with a history of bronchospasm. If Salmeterol and Fluticasone Propionate Pressurized Inhalation therapy has to be withdrawn due to overdose of the beta agonist component of the drug, provision of appropriate replacement steroid therapy should be considered. Additionally, hypokalaemia can occur and potassium replacement should be considered.

Acute: Acute inhalation of fluticasone propionate doses in excess of those recommended may lead to temporary suppression of adrenal function. This does not need emergency action as adrenal function is recovered in a few days, as verified by plasma cortisol measurements. Prolonged treatment of patients with high doses of inhaled corticosteroids may result in adrenal suppression and acute adrenal crisis. Monitoring of adrenal reserve may be necessary. In cases of fluticasone propionate overdose Salmeterol and Fluticasone Propionate Pressurized Inhalation therapy may still be continued at a suitable dosage for symptom control.

STORAGE
Store below 30°C. Do not freeze. Keep all medicines out of reach of children.

PACKAGING INFORMATION
Each sales pack is available as canisters containing 150 metered doses.

SHELF-LIFE
24 months

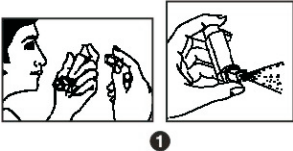
DATE OF REVISION: 12/04/2024

PRODUCT REGISTRATION NUMBERS
Combiwave SF 125:
MAL1405154AZ
BRU20012766P

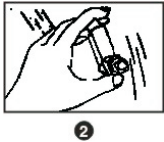
Combiwave SF 250:
MAL14125084AZ
BRU20012767P

INSTRUCTIONS FOR PATIENT FOR USING
THE INHALER WITHOUT SPACER
Important: Follow instructions carefully.
Shake the inhaler well immediately before each use.

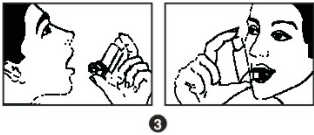
Testing your inhaler:
If you are using the inhaler for the first time or if the inhaler has not been used for a minimum of seven days, "test spray" the inhaler. Remove the cap from the mouthpiece; the mouthpiece should be inspected for the presence of foreign objects before each use. Spray the inhaler 2 times into the air after shaking the device prior to each actuation. (See pic no. 1)



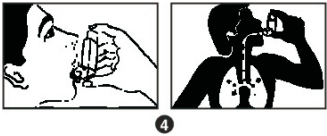
Make sure the canister is fully and firmly inserted into the actuator. (See pic no. 2)



Hold the inhaler upright with your thumb on the base. Place either one or two fingers on the top of the canister. Breathe out fully through your mouth expelling as much air from your lungs as possible. Thereafter, place the mouthpiece of the inhaler in your mouth between your teeth. (See pic no. 3)



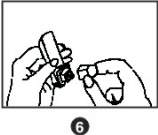
Close your lips around it (do not bite it) tilt your head slightly backwards. Start breathing in slowly through your mouth. As you breathe in steadily and deeply, press down the canister to release one puff. (See pic no. 4)



While holding their breath, patients should take off the inhaler from their mouth and should continue holding their breath for 10 seconds or for as long as it is comfortable. Breathe out slowly. (See pic no.5)



Note:
If the second dose is required wait for at least one minute and repeat steps 2 through 5 for each puff prescribed by your Physician. Rinse your mouth or gargle with water after inhaling your recommended dose (Single dose means total number of puffs advised by the Physician at once). This is likely to reduce the soreness that may be caused by the drug. After use, replace the mouthpiece cover. (See pic no.6)



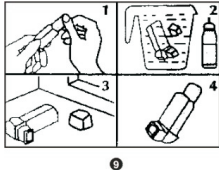
Practice in front of the mirror for the first few times. If you see 'mist' coming from top of the inhaler or sides of the mouth, this indicates failure of technique. Start again from Step 2. (See pic no. 7)



For Children:



Cleaning:
Clean the inhaler at least once in a week.
1. Gently pull the metal canister out of the plastic body of the inhaler. Remove the mouthpiece cover.
2. Rinse the plastic body and the mouthpiece cover in warm / running water.
3. Dry thoroughly. Avoid excess heat.
4. Replace the canister and the mouthpiece cover correctly.



Discard the inhaler along with the canister after using the labeled number of inhalation.
DO NOT PUT THE METAL CONTAINER IN WATER

Product Registration Holder:
Glenmark Pharmaceuticals (Malaysia)
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ICONGRAPHICS CODE:

PANTONE SHADE
PANTONE BLACK PROCESS C
Supersedes Artwork Code:

PHARMACODE: